

Mon, Jan 09, 2023 5:34AM • 46:27

Interviewer 00:00

Now and we are recording. All right. Now just to start, as well, I just wanted to get a better sense of your clinical practice. You primarily provide EFT emotion focused therapy. Is that right ? For couples therapy?

Therapist 00:20

Yes. Yes. Emotionally Focused couples therapy. I mean, I do some individual EFT, I do some family but 95% of my practice is couples.

Interviewer 00:34

Sorry Is that Emotionnally focused therapy, or Emotion Focused Therapy ?>

Therapist 00:38

Emotionally focused emotionally I worked with [name] for well over [16-20] years. So

Interviewer 00:47

Allright, and how long have you been working as a couple therapists now?

Therapist 00:54

Wow. I mean, I first started doing therapy in [year]. And I've worked with couples, you know, for the most part ever since then. There were a few. And then academics as well. But you know, most of that time.

Interviewer 01:17

And are there specific types of couples that you provide your therapy to?

Therapist 01:23

All types. These days, I work a lot when they're with affair couples. But that's just something I've fallen into. I mean, I work with or not.

Interviewer 01:34

All right. And you primarily provide your service to couples in the [Country A]? Am I correct?

Therapist 01:41

Correct.

Interviewer 01:42

All right. So I know there's a bound to be you know, like, of course, there's a broad range of issues. But when your clients come to you, what is usually the presenting issue, or the reason that they seek your services?

Therapist 01:57

Well, couples distress, I mean, you know, in various ways, it's almost always coupled distress. There's other of course You know, there are other issues, but it's almost always a couple of distress. There's addictions, there's, you know, depression, there's PTSD, there's variety, things like that.

Therapist 02:34

And do you do a formal assessment when they come in?

Therapist 02:30

No.

Therapist 02:32

Why not?

Therapist 02:34

Because I've learned over the years that I can typically get a lot more out of doing kind of an impersonal I mean, not impersonal, in person interview. And using by formal, I'm guessing you mean, using an instrument? I don't...

Interviewer 03:00

So, you said that you've learned over the years that it was better for an in person interview? Am I correct in assuming that you were doing that sort of formal instrument? Pen and paper assessment?

Therapist 03:13

Long ago I did. Yeah.

Interviewer 03:14

And how was that and why did you eventually make the shift to make the transition to just in person interviews?

Therapist 03:21

Um, I just found I learned a lot more. And the formal, you know, I guess it would be easier now. But kind of when I stopped doing them, getting them to instrument, having them spend the time filling it out, you know, for the majority for the most part, it wasn't worth the time and energy and just wasn't learning that much from them.

Interviewer 03:48

All right. And can I know what these instruments were before?

Therapist 03:53

I used to use the DAS Dyadic adjustment scale, the Beck Depression Inventory. The... and I, every once in a while I still use that one. Long ago, I use the Marital Status Inventory. I use a long ago I use the Lock Wallace. Marital just .. what was called I don't remember.

Interviewer 04:23

Did you pay for any of these scales?

Therapist 04:27

Just the Dyadic A Scale . No! Just the Beck Depression Inventory.

Interviewer 04:36

Do you mind if I ask how much that cost back then?

Therapist 04:39

I have no idea. I don't remember.

Interviewer 04:41

Okay, no problem. So you say

Therapist 04:44

The DAS is in the public domain. So..

Interviewer 04:47

Okay. You said that you weren't getting much out of it for you. Was it the time, the effort? Were there other challenges that you were facing when you were using the scales back then?

Therapist 05:04

Well, it was just the hassle of administering it. And sometimes people, I think oftentimes those instruments kind of reflected the emotional mood at the time they take them. I think there is people's views of their relationships are very state dependent emotionally state dependent has been my experience. And so if they've had a rough day or two, it's much worse, if they've had a better day, it's better not to say that there isn't value in them, certainly in research and, you know, involved in a research project right now where we are using them. So I certainly have used them over the years in research projects. But for clinical use, because I I'm a professor, I chair dissertations, I publish all of that. And but for clinical use, I just haven't had found it, interesting.

Interviewer 06:06

Interesting. And one of the questions here is how informative do you find the skills to be and for you wasn't informative. These DAS, the marital status inventory, marital interest? How informative do you find these ,when t you were using it as a practitioner?

Therapist 06:25

Right. Not that informative.

Interviewer 06:27

All right. And I also touched upon this and the email, but across the years, different therapists and different researchers have characterized the construct of relationship quality differently from each other. I wanted to get your view about it. For you, What is relationship quality?

Therapist 06:48

You know, I've thought about that, since you've said it. And you know, and you mentioned some kind of an international perspective on that. across nations, I know that's what you're looking into. And I, you know, I've trained therapists in many different countries over the years, and talk with many different clinicians, and I've done demonstration sessions in a lot of different countries. I think, you know, there are cultural ways of understanding satisfaction. Some people put in some cultures, there's a lot more emphasis on productivity in terms of economic productivity, particularly for men. And yet, I do think that ., I guess, would come from my EFT background, there is a scale called the BARE. The Brief Accessibility Responsiveness and Engagement scale, that I actually made a recommendation on a research project yesterday that they consider incorporating that one. I think that, you know, and this comes straight out of EFT, accessibility responsive engagement is helpful. I also think that there's things like the Kansas Marital Satisfaction, inventory. That's like pretty global. And I think that there's value in that way, because people kind of end up defining it the way they the way they would see it, what's important to them. I do believe that there are differences between the way people establish whether they're happy or not. And the way that researchers are able to predict and conceptualize distress. And sometimes I think that being I was deeply involved in John Gottman observational research early in my career is an observational coding study using one of his coding systems, which, you know, he's been able to predict wars in some of that early work very highly. But behavior I mean, he was looking a lot at behavioral message. But I think sometimes predicting Divorce isn't necessarily the same or satisfaction. Still ability isn't necessarily the same thing as the way people are going to define whether they're happy in their relationship or not. When people can be unsatisfied and still highly likely to stay together. That makes sense because stability and satisfaction are two different things. Your question, though coming back to your question is about defining satisfaction. Right? Not stability.

Interviewer 10:34

My question was on relationships quality yes, how you would conceptualize how you conceptualize relationship quality?

Therapist 10:41

Quality is the word you used and I probably shouldn't try to write some of this out beforehand, but I thought I'm going to talk with you and get a better sense of what you're after. I mean there are elements of quality, that are key in terms of okay, I think a quality relationship is one that is stable, okay, people are not ready to jump out at anytime. So there is an element of commitment and quality relationships are relationships where people can find physical and emotional safety in the relationship. They can experience the relationship as kind of a safe haven from the storms of life. In other words, couples, either implicitly or explicitly can turn to the other for comfort and safety. That's a huge thing. Quality relationships also are very good at repairing. You know, because in almost all all relationships, all relationships, there's breaches of connection and quality and quality relationships. Those breaches, couples are very good at repairing the breaches. Okay. So I think, let me just backtrack on what I'm

saying here, breaches, you know, being able to repair while your relationships people are good at repairing quality relationships, people can find safety and connection and physical emotional safety and connection that can be soothed, comforted, quality relationships, people also can find enjoyment with the other person and that enjoyment and we be both in doing things, but also in kind of enjoying some of the differences. you know, Gottman talks about how it's not the number of differences that relationships are in incompatibilities, quote unquote. It's the way couples handled them. And sometimes couples that are very different can really enjoy their relationship by admiring and sprek and, and respecting and even find humor around some of their differences. So anyway, they can enjoy the relationship. For some couples that can be having doing a lot of things together for other couples. They may not as have as many over kind of arching activities that they enjoy doing together, but they find other aspects of the relationship that they can find enjoyable

Interviewer 14:44

Interesting All right, thank you so much for that answer. So for you, what differentiates couples with higher relationship quality and those with low relationship quality?

Therapist 15:04

Relationship Quality couples as opposed to low, high relationship qualities, or relationship couples, those who they are able to find joy and happiness in the relationship. They as opposed to those who are distressed do not. Those who are distressed tend to get caught in negative patterns of interaction, they get reactive with each other, they follow somewhat predictable patterns, expressing reactive emotions becoming they tend to not share their more vulnerable emotions, they tend to be more likely to be critical, they tend to focus on what goes wrong, as opposed to how to improve things. Often blaming focus on who did what was what is wrong, you know, rather than focus when something goes wrong, how to prevent it or change it. Or make it so that it's not a big issue. happy, successful quality relationships, as you said, they tend to be much better at focusing themselves on something goes wrong, somebody gets hurt, it's not about blaming, as much as it is about, you know, focus on repair and prevention. Which tends to wear loose, you know, tends to and they focus on comforting, they focus on trying to comfort each other. In happy quality marriages, they tend to kind of there's a good deal of acceptance of where the other person is. There's less of a focus on changing the other person, but there is oftentimes a focus on bringing about a change and each person actively bring about a change to improve and prevent problems.

Interviewer 17:36

All right. So in your professional experience, what are the most important things in a high quality relationship?

Therapist 17:47

I thought I answered that one.

Interviewer 17:49

All right. No problem with that.

Therapist 17:51

I mean, I can go back over the same things I just said.

Interviewer 17:54

Sure, no problem.

Therapist 17:58

What are the things in quality relationships? Hmm. Couples have a shared identity. And I mean, in some of garments research just in his, his relationship history, and I will do questions of history. And I do pay a ton of attention to some of these factors. More often they use the word we and talking about their history as opposed to I, the happier they seem to be. And I, to me, that represents a shared identity. Where they actually are seeing themselves as a part of a unit. Their partner? Yeah. As opposed to an "I" that's trying to maximize their own happiness. It's a we and their identity shifts. We and "us" kind of identity. And yeah.

Interviewer 19:15

So you think a lot about this relationship quality, the difference between like high quality and lower quality relationships, but I wanted to know like, that's some way that you can separate relationships often differ from how your clients conceptualize relationships? And did that impact the therapy on how you do therapy?

Therapist 19:41

Clients often come in, in pain or hurting. They do at times have, very usually they themselves have different conception of what a happy relationship looks like. And part of the work is helping them find a destination that works well. And I do play an active role in helping that happen. But ultimately, it's something that they themselves co construct. But I'm assisting, I do think that there are processes that do tend to make a difference. I mean, in happy relationships, people tend to be emotionally open and responsive. And whatever it looks like, however, amount of closeness or distance or, you know, some of the markers, there is the ability to turn to each other for comfort, there is the ability to be emotionally open and vulnerable with each other. And so I think those are kind of general, they may look really different in different cultures. Kind of the processes, do you look in the eyes? Do you look down? Do you have, you know, traditional patriarchal kinds of hierarchy or not, but some of the rules of society are different. But as a key element emotionally connected, vulnerable, and there's a certain amount of it, you don't have to be vulnerable all the time. It's just, it's kind of like, there has to be sufficient. And sufficient, maybe higher than others. But I do think so I think there's elements of, okay, this is where we want to be this is how we see ourselves as a couple, or where we want to go. And they're often different. Sometimes I have people sculpt this to try to make it more explicit. And there's a nail there's that piece, and I also think there's some universals.

Interviewer 22:23

All right, I wanted to ask you as well, [participant] that you know, like the relationship is a complex system. It involves love, sex, identity, relationship confidence and conflict, satisfaction, all of that. In therapy, how do you decide which one to focus on? How do you decide which one in this complex system to target?

Therapist 22:49

Um, well, I typically work with what's alive in the room. Okay, where the pain points are. And because the pain points tend to drive to negative cycles, that couples get caught in and so I'm working with pain points, sex usually is dependent, good sex tends to be dependent on safe emotional connection. So couples that come in with, you know, very distressed sexual lives, their difficulties, they're not having sex at all. Somebody's usually mad about that. There's a pain point but I really work on connecting that I don't do sex therapy per se at that point. It's too early on. I work on identifying what that symbolizes. Let me just say something I do think that most problems, most pain points in couples are related to people feeling like they don't matter. Mm hmm. Sue Johnson says the \$94 million question that underlies almost all couple conflict as "Do you matter"? Right on that am I important to this person that I love when that is in place? Differences around sexuality, traumas, health problems, couples can navigate. When couples fundamentally feel or remember if one in a couple find feels like they don't matter for, it's difficult, it's super difficult. So I guess going back to to your earlier question about relationship quality, a quality relationship have as a characteristic, both members of the couple felt like they really mattered to the other person. It ties in with safety, which I talked about. But they feel like they really matter. And not just I mean, their feelings, their hopes, their dreams, their desires they matter, doesn't mean that everything goes their way. Mm hmm.

Interviewer 25:30

Interesting. So then when do you determine when to end the couples therapy?

Therapist 25:36

To end couples therapy?

Interviewer 25:39

That's right.

Therapist 27:42

Well, I look for some markers. One marker is couples are able to stay out of negative cycles effectively. More importantly, when they are distressed, they can reach and a vulnerable and effective way for comfort to their partner, once they can really reach in a vulnerable and effective way get comfort from their partner. Typically, they can navigate the storms of life. And so that's a key marker means sometimes people will want to stay on for I don't know, to provide to have the safety of the session provide, you know, help them navigate some big problem, that's been a huge thing. And, and I will do that, but I start working toward termination when they I see them consistently reaching toward each other. And usually they come in and they're they're much happier. They feel more secure and safe.

Interviewer 27:04

I know again, there's gonna be a broad range here, but typically how long from start to finish that usually take that they've been able to get these key markets.

Therapist 27:14

Broad, broad, broad, broad range. All right. Sometimes it's 12 sessions, sometimes it's years and years.

Interviewer 27:30

And how long are these sessions?

Therapist 27:34

The sessions that I do are typically an hour and 15 hour and 20 minutes,

Interviewer 27:38

every week, every other week, typically, every

Therapist 27:41

week, I will oftentimes move toward it every other week as they get better.

Interviewer 27:52

All right. So as you know, from the email, we are currently developing a relationship quality scale that hopefully better represents our relationship as a complex system that could hopefully help therapists and treatment planning and evaluation. How interested would you be in a scale like this?

Therapist 28:13

Well, I think I would, I'd have to look at the questions. But I would and I think particularly if it was had questions around mean, things like, you know, how often can go to your partner with your pain or your hurt or your fears? Is your partner responsive? You'll see some of those kinds of questions on the bear the response on the casement questions around identity. I think that would be interesting to know, you know, how much they see themselves as a way you're supposed to. And I I think that questions around kind of resolving painful issues. You know, are there major issues that are unresolved? I think that that are still causing pain, not talking about, you know, how good are they at avoiding conflict and way of avoiding conflict? Maybe not the right word. But are they feeling like they can reach some resolution around conflict, so the conflict doesn't define them. All right.

Interviewer 29:38

Okay. Anything else that would make this more useful for your practice? If ever?

Therapist 29:50

You know, if I were to develop a scale maybe this is the Richards or in me, but I think I would look at questions from other scales. And that's often done. But I'd look at questions from other scales. And I would also kind of look at what I found most useful in therapy. What are some of the things that we've talked about? And there may be questions, because as you know, the way a question is worded is hugely important, has a bigger answer. Yeah, I mean, I think I would do that. And I'd also just think about, it would need to be reasonably short. Some of the most important pieces

Interviewer 30:51

isn't short. What is the reasonably short for you?

Therapist 30:58

I'm not sure.

Interviewer 31:02

I was 20 minutes too long.

Therapist 31:06

That would be the outside of too long, I would say, you know, or less? In part, because I bet this is for clinical use. It's not for research. Although I think a lot of times there, there's a lot of overlap there. In part, because it's the answer to the questions that's most important, not the scoring. You could score really low on the DHS, for example. And, okay, they're highly distressed and highly unhappy. But it doesn't give me the nuts and bolts of what's going on. Right, you can score. I'm much more interested in on an overall score as a researcher. But as a clinician, I am much more interested in, what is this going to tell me that's going to make my assessment go faster? Maybe point me on issues. I know, I probably want to ask something about secrets in a general way. Mm hmm. That's another thing that I would find useful. Are there things that I have done that I'm ashamed of, and I want my partner to know about? scale of one to five, you know, in part because those oftentimes shame is a block it create blocks to intimacy, and I need to explore that with people.

[Portion Redacted]

Interviewer 34:32

So, of course, you're already contributing to the development of the scale just through this interview, but I need to ask as well, anyway, would you be interested in further collaborating for the development of the scale?

Therapist 34:44

Actually would

Interviewer 34:47

perfect All right.

Therapist 34:49

Yeah. Let's just because there's a lot and just our interview, again, it's had me thinking about these issues, and I'll probably think about this more Again, your email was for me. It seemed very, very broad. The but I would be interested in it, because I think developing a decent scale. I mean, if I were to use anything, I'd probably be the bear. But I'm not that. I mean, the bears got its issues. Okay.

Interviewer 35:22

All right. Actually, we're wrapping up. So I just like to review what we've discussed over the course of the session. You started out with your clinical practice, he would do emotion and D focus therapy since 1989. Couples of all types.

Therapist 35:37

Yes, but I have not been doing Emotionally Focused Therapy since [year]. I got involved in Emotionally Focused Therapy in [year]. Before that's what

Interviewer 35:52

before before being emotionally focused, what were you doing?

Therapist 35:58

The area's systemic. I mean, I used a variety of different models, because I was learning to be a therapist. Right, I started out with the old what was then called behavioral marital therapy, Jacobson, and Margolin 1979. I certainly spent time with strategic therapy. I did work with [name] for about a decade, which was interesting. Although by then I was really into Emotionally Focused Therapy. I spent my time with solution focused therapy. I spent time with Bowen, family systems. And structural, probably more structural for family, my family work less for my couples work. But I experimented and tried on different models. Really, before I got into Emotionally Focused Therapy,

Interviewer 36:55

and you provide therapy to couples of all times within the [Country A], usually, I believe you said heterosexual couples, typically,

Therapist 37:03

most of them are set up for sexual couples. Yeah. All right. And I do an enormous amount of supervision to a lot of couples therapists. I'm an EFT trainer and supervisor. And so I just see a lot of couples, you know, in various you know, sexual minority majority. All right.

Interviewer 37:31

When clients come to you, it's usually very responsive to cover that stress. You know, there might be other issues, addiction, depression, PTSD, but it's almost, it's almost always coupled with stress. You don't anymore do a final assessment. You used to do it earlier in your career. You think that it's not worth it, it's not worth the hassle of administering it. I used to use a s Beck Depression Inventory, marital status inventory. Lock, while what causes American richest man, but for you didn't get you didn't find it too informative. And you find it all like in person interviews much more informative. So yeah,

Therapist 38:11

I probably early in my career, I also use the MCM. I remember that scan score was a moral psychological test. Anyway, keep going.

Interviewer 38:23

All right, for the relationship quality for you. Earlier on, you mentioned really like satisfaction, even though for example, like culturally across the world, there might be different ways of like expression, etc. But you were talking about, you know, people, there might be differences in how people establish if they're happy or not. But you were talking about satisfaction. And then you got around to saying about other elements of relationship quality, like physical and emotional safety, like stability and commitment. Like they're aware of their relationships. So if there's breaches of connection, quality relationships are being able to repair them to beaches, you're also talking about like the enjoyment again, a satisfaction or the enjoyment with other person either both doing things or enjoying the differences. So for you what differentiates Hey,

Therapist 39:17

did you did you mention safety,

Interviewer 39:20

safety, physical and emotional safety? And for you what differentiates got worried with high and low? Sorry, go ahead,

Therapist 39:28

you're able to start to feel valued, like you matter, and to be able to reach for comfort, and whatever? Yeah, I just want to I know, I answered those at different times.

Interviewer 39:43

And I know that you said that feeling that you're valued and that you matter, you connected with emotional safety. All right, and for what differentiates couples that high quality relationship and low quality relationship for you is just really being able to find joy and happiness for this day. In distressed for you, those has no relationship quality, or those were like negative patterns of interaction, they tend not to share their vulnerability, they're more critical. And the happy, successful relationships tend to focus more on bettering themselves instead of blame, instead of criticizing, they able to comfort each other accept each other less about changing the other person, but bringing change to their relationship to prevent and repair conflicts. They all you also mentioned about having a shared identity, the more use of the word we and I that they, they feel like they're in the unit. When talking about the different conceptualization, between you and the clients, for you that like, that's part of therapy, or like different, their CO construct where the destination has to take an active part of of that, but it's ultimately something that is CO constructed. And, yeah, but there are for you like, even though there are, you know, like cross cultural differences for you, there are universal things that matter, for example, you if you're more emotionally responsive, if you're connected, if you have sufficient vulnerability that are important for you, universally, even though some other cultural expressions might differ, like how sufficiently vulnerable that is, differ across cultures, and so on. And when asked about how to target in within the complex system for you, it's usually what's alive in the room, the pain points, I believe that's usually you know, taking cue from, from what the patient's itself themselves bring up in the room. So for you things like for example, sex are dependent on intimacy or safety of emotional connection. So, and then you also emphasize really, really, really that what they should know whether you agree with the Johnsons \$94 million question about whether they do matter. So for you do to determine the end of couples therapy is to look for markers that they're able to stay out of negative cycle that the when they're distressed, they're able to reach a vulnerable and effective way to get comfort out of their partner. So you think that if they're able to do this, and they can navigate the storms of life, as you were saying, broad range 12 sessions, to years, and years and years, 1.51 point 15 to one point 20 hours every week or every other? You are interested at the scale development? Particularly you want questions around how often can you go to your partner with your pain, are passionate about identity with a few questions about how to resolve or what how they resolve painful issue. There's questions about secrets as well, if there are things that they've done, that they're ashamed to let their partner and you want it to be reasonably short, 10 minutes or less about you're not actually sure for research, clinical use.

Therapist 43:06

And for clinical, I think a lot of it is I mean, if it's done online before therapy, it's 20 minutes is fine. I just don't want to take 20 minutes reviewing it, or 40 minutes reviewing it, you know, one for each person. That's way too much for me, you know, I have a session. And so it's more I think about the amount of time I'm going to I want to to hit I want to be able to hit the high points. I don't want to spend 40 minutes going over two of them.

Interviewer 43:40

Perfect. All right, I can I can imagine how that would look like. And then you're interested in collaborating, scale. Anything more you'd like to add on anything you've thought about. All right. Now before we end, I'd like to talk to you about how we will handle the data. Of course with open science, we want our research project to be as open as ethically possible. And you know, to be as transparent and verifiable. So I wanted to know which data sharing option you were more comfortable with. One option is that the transcript portions can be shared, none of which can be identified to you. But if you were bolder, another option is to share the whole transcript removed of sensitive and overly identifying information. So in this way, like readers can go to the primary source and like get more context instead of just skip portions, although in this option,

Therapist 44:42

understand because what's your plans for these transcripts?

Interviewer 44:47

For example, if we were to if you are comfortable with sharing the transcript, anonymized transcript then it will be added as a supplementary material for for the article so it will We save all of our claims. And then you can say, of course, you don't have a problem with that. Although I need to say, of course, the risk is that someone who knows you professionally, well, might piece together details in the transcript and narrow it down. Alright, so I'll be giving you

Therapist 45:18

my name ends up coming up, that's fine.

Interviewer 45:23

Yeah, well, either of these lots. And if there's anything you want through that, that

Therapist 45:29

I don't think that there's gonna be anything that needs to be redacted. I would say. The only thing is, is that, you know, I do jump around here a bit. And, you know, my thinking on this is developing as we're talking. So in some of your later questions, I was really kind of responding to some of your earlier questions. And that is, I think, if anything that could be misleading to anybody. It would be okay. You asked a question. I went through several thoughts, questions, and then you went through another question. And then all of a sudden, I'm jumping back and I wanted to add there. That could be just somebody just needs to read the whole thing and not rigidly put my answers into one. All right. I don't care if my name is associated with

Interviewer 46:24

Okay, thank you so much.